INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Nivolumab and ipilimumab for pleural mesothelioma

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00792
Version 2

Treatment stages

01 **Dual immunotherapy**
Dual immunotherapy

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

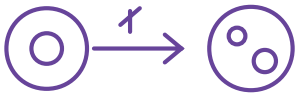
Three medicines. Different jobs.

This treatment aims to control pleural mesothelioma that cannot be removed with curative surgery, help symptoms and extend life. It is usually given without an expectation of cure. Mesothelioma starts in the lining around the lung and is different from non-small-cell lung cancer.

IMMUNOTHERAPY

Nivolumab

Opdivo



Blocks the PD-1 immune checkpoint

Nivolumab binds PD-1 on T cells. It prevents signals that can reduce the immune attack on cancer. This can also let immune cells inflame healthy tissues. It is infused every 21 days in this plan.

T cell → PD-1 blocked → Immune response

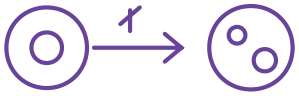
What to watch for

New breathing, bowel, skin, hormone or other symptoms need prompt advice during or after treatment.

IMMUNOTHERAPY

Ipilimumab

Yervoy



Blocks the CTLA-4 immune checkpoint

Ipilimumab blocks CTLA-4, another control on T-cell activity. It works alongside nivolumab through a different immune checkpoint. It is given every 42 days, so it is absent from each Day 22 visit. Healthy organs can also become inflamed.

T cell → CTLA-4 blocked → Immune activity

What to watch for

Report diarrhoea, tummy pain, rash, severe tiredness, headache or other new symptoms promptly. Keep your immune alert card.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why this treatment is offered

What does the diagnosis mean?

Pleural mesothelioma begins in the lining around the lung. The team confirms the subtype and that curative surgery is not suitable.

What are the alternatives?

Ask about other systemic options, trials and symptom-focused care in the context of your cancer, health and priorities.

Why this treatment is offered

In CheckMate 743, nivolumab plus ipilimumab improved overall survival compared with platinum/pemetrexed chemotherapy in previously untreated unresectable pleural mesothelioma.

Trial results compare groups and cannot predict your personal benefit. Discuss how the study population relates to your cancer and health.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Induction → review → maintenance

Nivolumab and ipilimumab for pleural mesothelioma

42-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DUAL IMMUNOTHERAPY

Dual immunotherapy

D1 D22

Until progression, unacceptable toxicity or up to 24 months without progression. Each cycle lasts 42 days: nivolumab and ipilimumab on Day 1, then nivolumab alone on Day 22. Nivolumab is every three weeks and ipilimumab every six weeks. This regimen contains no chemotherapy.

Maintenance means ongoing treatment, not treatment without side effects. Your plan can change after discussion.

BASELINE → TREATMENT → FOLLOW-UP

Checks throughout treatment

Relative stages; no dates need to be entered. Ask who arranges and reviews each check.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00792 v2 · official regimen

THE WHOLE TREATMENT JOURNEY

Nivolumab and ipilimumab for pleural mesothelioma: treatment and checks

First-line unresectable malignant pleural mesothelioma in adults.

Each cycle lasts 42 days: nivolumab and ipilimumab on Day 1, then nivolumab alone on Day 22. Nivolumab is every three weeks and ipilimumab every six weeks. This regimen contains no chemotherapy. Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Treatment NCCP 00792	Confirm indication and plan	Nivolumab + ipilimumab	Nivolumab alone	Nivolumab + ipilimumab	Nivolumab alone	Agree stop plan and continuing care

Care / check	Before starting	Cycle 1 · Day 1	Cycle 1 · Day 22	Later cycles · Day 1	Later cycles · Day 22	Completion / follow-up
Blood count, kidney and liver NCCP 00792	Baseline results	Review before treatment	Clinical review; additional tests as indicated	Review before every cycle	Clinical review; additional tests as indicated	Follow unresolved toxicity
Hormones and immune symptoms NCCP 00792	Glucose, thyroid function tests (TFT); hepatitis screen; cortisol if indicated	Glucose/ TFT before cycle and symptom review	Review symptoms; extra tests if needed	Glucose/ TFT before cycle and symptom review	Review symptoms; extra tests if needed	Vigilance at least five months after last dose
Support medicines NCCP 00792	Carry treatment details and the immune alert card. Describe your usual bowel pattern, breathing, energy, skin and hormone symptoms, and tell the team about autoimmune illness, infections and all medicines.	Low-risk visit support	No routine minimal-risk prophylaxis	Low-risk support; no chemotherapy prophylaxis	Minimal-risk support; no routine prophylaxis	Confirm medicines and steroid taper if used
Response and preferences NCCP 00792	Discuss goals and alternatives	Ask about symptoms and priorities	Review treatment experience	Consultant-directed scans and benefit review	Report new concerns between visits	Agree responsible team and symptom support

What each check is for

1 **Baseline**

Before first infusion

Confirm exact eligibility

Ask / act: Review results, consent, medicines and contacts.

2 **Day 22**

Three weeks after Day 1

Different active medicines

Ask / act: No ipilimumab on Day 22; confirm support plan.

3 **Continuation**

Later 42-day cycles

Review benefit and toxicity

Ask / act: Apply joint immune hold rules and review duration.

4 **End of treatment**

At progression/toxicity or up to 24 months

Late immune effects can occur

Ask / act: Document follow-up and urgent contacts.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Visits and duration

STEP 2

Time in the unit

Nivolumab over 30 minutes, then ipilimumab over 30 minutes on Day 1 only. Ipilimumab vital signs are checked every 30 minutes during infusion and for an hour afterwards. Blood tests, preparation and observation add to time in the unit.

STEP 3

Medicines and monitoring

Blood count, kidney, liver, glucose and thyroid tests before each 42-day cycle; symptoms reviewed before each infusion and additional tests when indicated. Cortisol if clinically indicated. Carry treatment details and the immune alert card. Describe your usual bowel pattern, breathing, energy, skin and hormone symptoms, and tell the team about autoimmune illness, infections and all medicines.

STEP 4

Transport and support

Arrange travel and help if needed. Keep urgent contacts and alert information with you.

Before the first visit

Your usual health

Describe usual bowel movements, breathing, energy and skin; report autoimmune illness, infection, nerve symptoms and every medicine or supplement.

Practical preparation

Arrange blood tests, transport and support at home. Keep treatment names, urgent contacts and your immune alert card available.

Checks before and during treatment

Before starting

Baseline full blood count (FBC), renal/liver profile, glucose and thyroid function tests (TFTs); hepatitis B surface antigen and core antibody and hepatitis C RNA. Confirm no prior chemotherapy for pleural mesothelioma.

New symptoms

Extra tests may be needed between visits. Routine blood results alone cannot exclude every immune effect.

Response

Scans follow the consultant plan; discuss benefit, uncertainty and what would lead to changing treatment.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Day 1 visit	 **Day 1** Includes nivolumab and ipilimumab
Day 22 visit	 **Day 22** Nivolumab; no ipilimumab
Immune symptoms	 **Days 1–42** Any day, including after treatment stops

Treatment schedule: Each cycle lasts 42 days: nivolumab and ipilimumab on Day 1, then nivolumab alone on Day 22. Nivolumab is every three weeks and ipilimumab every six weeks. This regimen contains no chemotherapy.

No guaranteed symptom-free days: Side effects vary and immune symptoms can start after treatment stops.

Duration: Continue while appropriate until progression, unacceptable toxicity or up to 24 months without progression. Your team reviews the plan and may delay or stop treatment.

After treatment: Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

> **Why these bands stay broad:** 42-day reference cycle with different Day 1 and Day 22 medicines. Symptom bands are not a prediction of recovery. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.

> **There are no guaranteed "safe days".** Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Between-treatment notes

Day 1: Day 1 visit

Nivolumab and ipilimumab are due. Report symptoms and confirm the current prescription.

Day 2–21: Between visits

Follow your prescribed support plan and report new symptoms promptly. Immune effects can occur on any day. Keep the oncology contact instructions and immune alert card accessible.

Day 22: Day 22 · no ipilimumab

Nivolumab is due. Confirm active medicines and the right support plan.

Day 23–42: Before the next cycle

Continue symptom reporting. Later Day 1 visits again include both immune medicines if prescribed. Confirm the next appointment and blood tests; do not wait for it if unwell.

Treatment effects

Bowel, lungs and skin

New diarrhoea, tummy pain, blood/mucus in stool, cough, breathlessness, blistering or peeling skin need prompt assessment.

Hormones, heart, muscles and nerves

Unusual weakness, headache, dizziness, marked thirst, frequent urination, chest pain, palpitations, drooping eyelids or difficulty swallowing/breathing need urgent advice. Several organs can be inflamed together.

Liver and kidneys

Yellow eyes or skin, dark urine, reduced urine or swelling need prompt review; blood tests can find changes before symptoms.

Support medicines and self-care

Steroids may be prescribed to treat immune inflammation. Follow the exact dose and taper; do not start, stop or change them without your team. Hormone replacement can sometimes be long term.

When unwell

Follow your oncology emergency instructions for fever, shivering or sudden illness even with a normal temperature. Severe breathing difficulty, chest pain, collapse or confusion needs emergency care.

After treatment and daily life

Reproductive advice

Discuss pregnancy, breastfeeding, fertility and contraception before treatment. Effective contraception is needed for at least five months after the last nivolumab dose; ask about your individual plan.

Living with treatment

Ask for help with fatigue, appetite, pain, breathlessness, work and emotional concerns. Follow-up continues for unresolved effects and any long-term hormone replacement.

Questions worth asking

What if I miss a support medicine?

Ask your oncology team or pharmacist for advice; do not double doses or combine steroid courses yourself.

How is benefit checked?

Your consultant sets the scan plan and reviews symptoms, results and priorities. Side effects do not reliably show cancer response.

Can I decide from this guide whether treatment is safe today?

No. The team must assess symptoms, blood results, previous reactions and the prescribing protocol.

Try explaining the plan back: Which medicines are you having now? What changes after induction? What would make you call today? Ask your team to fill any gaps.

Nivolumab

- Immune inflammation in multiple organs
- Infusion reactions
- Delayed effects after treatment stops

Ipilimumab

- Immune bowel, liver, skin and hormone inflammation
- Other serious organ inflammation

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Every 42-day cycle · Day 1 nivolumab + ipilimumab

Active medicines: nivolumab + ipilimumab

Emetogenic risk: low

Medicine	Dose / route	When
Ondansetron	8 mg orally	Day 1 before treatment, if following the NCIS low-risk default Individualise/local policy. No automatic antiemetic corticosteroid course.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days as prescribed Specify duration and check contraindications/interactions.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Every 42-day cycle · Day 22 nivolumab alone

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Both immune medicines can inflame healthy organs during treatment or after it stops. Keep your immune alert card and report new symptoms promptly, including for at least five months after the last dose.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Nivolumab and ipilimumab for pleural mesothelioma

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Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hormones / brain

Nivolumab / ipilimumab

Unusual weakness, headache, dizziness, thirst or passing much more urine.

2 Lungs

Nivolumab / ipilimumab

New cough or breathlessness needs prompt assessment; infection or cancer can cause similar symptoms.

3 Heart / muscles

Nivolumab / ipilimumab

Chest pain, racing heartbeat, fainting or new muscle weakness needs urgent assessment.

4 Liver

Nivolumab / ipilimumab

Yellow eyes/skin, dark urine or abnormal tests can signal inflammation.

5 Kidneys

Nivolumab / ipilimumab

Kidney tests matter; report less urine, swelling or difficulty drinking.

6 Bowel / mouth

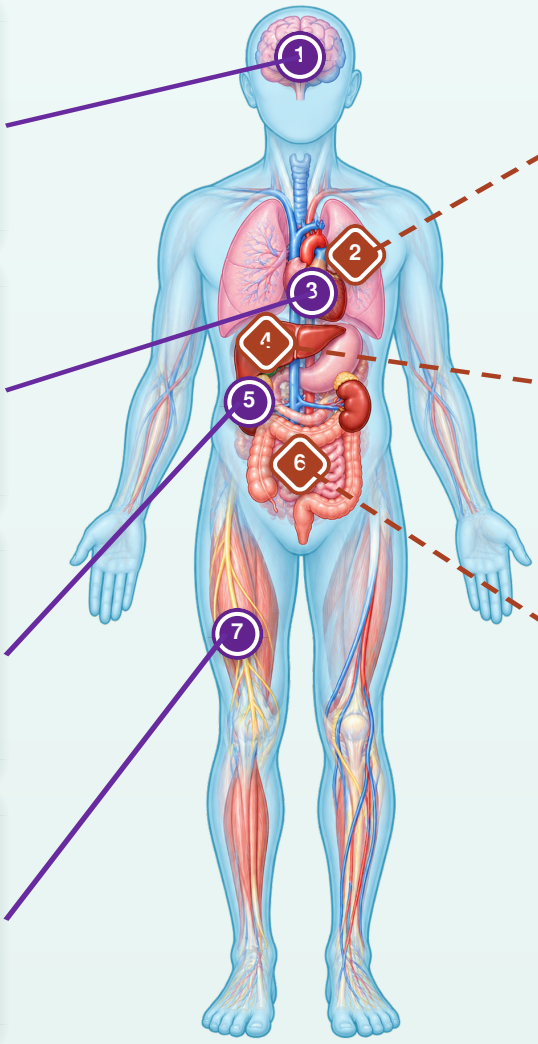
Nivolumab / ipilimumab

Report new diarrhoea, pain, blood/ mucus or mouth ulcers; do not assume ordinary chemotherapy upset.

7 Skin / nerves / muscles

Nivolumab / ipilimumab

Report spreading rash, blistering, numbness, weakness or difficulty swallowing.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Follow your oncology team's emergency instructions for fever, shivering or sudden illness, including with normal temperature. Report new breathlessness/cough, diarrhoea or abdominal pain, reduced urine, severe rash, unusual weakness, headache or marked thirst promptly. Severe chest pain, collapse, confusion or major breathing difficulty needs emergency care.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00792 v2 · official regimen**
   https://healthservice.hse.ie/documents/6873/792_V2_Nivolumab_360_ipilimumab1mg.kg.pdf
2. **CheckMate 743 — five-year outcomes (2026)**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC13001902/
3. **EMA Opdivo intravenous (IV) product information**
   https://www.ema.europa.eu/en/documents/product-information/opdivo-epar-product-information_en.pdf
4. **EMA Yervoy product information**
   https://www.ema.europa.eu/en/documents/product-information/yervoy-epar-product-information_en.pdf
5. **NCCP NCIS antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf